

HealthConnect Clinic

Project Integration & Coordination Report

Project Management Track - Week 6

Prepared by: Project Manager | Reporting Period: Week 6 | Status: Planning baseline signed off; Phase 3 In Progress

Week 5 → Week 6 Transition

- Main Week 5 output: the full Planning stage - WBS, dependency-driven schedule, network diagram, critical path analysis, schedule risk analysis, Resource/Communication/Quality Management Plans, cost baseline, and expanded Risk & Dependency Registers - plus Week 5 tracking (Task Tracker, Updated Timeline, Progress Report).
- Most important result: the critical path analysis, which showed the project's realistic duration is ~24 weeks and identified Phase 5 (Model & Assistant Development) as the phase with zero schedule float.
- Main issue/limitation discovered: three open items carried into Week 6 - data-quality severity was not yet validated (R1/R11), the IT Lead capacity conflict was unresolved (R12/DEP-009), and the Sponsor had not yet signed off the new schedule baseline (R13/DEP-010).
- Tracks relevant to Week 6: Data Analytics (to validate the data-quality finding), Generative AI (to close the Knowledge Base scope dependency), and IT (unresolved capacity conflict).
- What this week improves, integrates, or validates: closes out the three open items above, produces a Workstream Integration Map, and stands up the integration tracking infrastructure (issue log, decision log, coordination record) that Phase 3 onward will run on.

Part 1 - Week 6 Integration Readiness

A. Integration Point

- Tracks worked with: Data Analytics and Generative AI (two separate integration activities - see Part 3 for full detail on each).
- Output needed from Data Analytics: validated findings on the severity of the data-quality issue flagged in Week 5 (R1/R11), and confirmation of whether a workable patient identifier exists (DEP-002).
- Output needed from Generative AI: Week 6 safety and scope testing evidence, sufficient to close the Knowledge Base scope dependency (DEP-004).
- Output provided to those tracks: an updated Risk Register and Dependency Register reflecting their findings, and a formal Project Manager decision (logged in the Decision Log) on how each affects the schedule and scope going forward.
- Why relevant: both close loops opened in Week 5 that were blocking a clean, unambiguous start to Phase 3 and to assistant design work.

B. Integration Objective

After this integration: the Risk Register reflects validated (not merely suspected) severity for the data-quality risk; the patient-identifier dependency has a concrete, adopted resolution path rather than an open question; and the Knowledge Base scope dependency is closed rather than carried forward as an ambiguous in-progress item.

C. Evidence

- Updated Risk Register - R1 downgraded from High to Medium likelihood; R11 closed (merged into R1); R4 downgraded following accepted safety evidence; R13 closed (baseline accepted).
- Updated Integration/Dependency Register - DEP-002 moved to Mitigated; DEP-004 and DEP-010 moved to Complete.
- Decision Log - DEC-01 through DEC-04, each tracing a specific integration outcome to a specific decision.
- Cross-Track Coordination Record - four logged touchpoints this week, each with an outcome and a piece of evidence, not just a conversation.

Part 2 - Project Integration & Coordination Package (Tasks 1–13)

1. Workstream Integration Map

See HealthConnect_Workstream_Integration_Map.png. It shows how Data Analytics, Data Science, ML Engineering, and Generative AI connect to each other and to Project Management, and how Project Management in turn connects to the Sponsor/IT/Compliance/Clinic staff. Solid coloured arrows mark the integration flows that were actually active this week; dashed arrows show the standing PM coordination channel that runs regardless of week.

2–4. Dependencies, Readiness, and Further Development

Full detail is in the Integration/Dependency Register (HealthConnect_Week6_Integration_Registers.xlsx). In summary:

- Ready for integration this week: Data Analytics' validated data-quality findings (closed into the Risk Register), and Generative AI's safety/scope testing evidence (closed DEP-004).
- Requiring further development: the Data Science → ML Engineering model interface (DEP-011) is not yet defined and cannot be until Data Science's model comparison work completes in a later cycle; this is logged as a forward-looking dependency, not treated as overdue.

5 & 12. Tracking and Coordinating Cross-Track Activity

Four coordination touchpoints were logged this week - see the Coordination Record tab. Each has a topic, an outcome, and a specific piece of evidence rather than just a note that a conversation happened, per the assignment's requirement that communication alone does not constitute integration.

6. Integration Issue Log

Four issues were logged this week (ISS-01 to ISS-04); two resolved (the identifier gap and the Knowledge Base evidence format), two remain open (the Data Science/ML Engineering interface gap, and the IT Lead capacity conflict). Full detail in the Integration Issue Log tab.

7. Decisions Made This Week

Four decisions are logged in the Decision Log: accepting the 24-week baseline (DEC-01), adopting the fallback patient identifier (DEC-02), escalating the IT Lead conflict to the Sponsor (DEC-03), and accepting the Generative AI track's safety evidence (DEC-04).

8. Alignment with the HealthConnect Objective

Every integration activity this week traces back to the central question - reducing missed appointments and improving patient support. Validating the data-quality finding protects the model's eventual reliability; closing the Knowledge Base dependency keeps the patient assistant safely scoped; and escalating the IT Lead conflict protects the deployment timeline that the pilot and production rollout depend on. None of this week's activity expanded scope beyond the Charter.

9. Updated Project Timeline

The Planning phase gate (2.10) is now marked Achieved - the Sponsor signed off the baseline at the start of this week. Phase 3 (Analysis & Data Preparation) has moved from Not Started to In Progress. No other phase dates changed. See the Updated Timeline tab for full detail.

10. Risk and Dependency Review

The Risk Register now carries 14 items (R1–R14): three closed this week (R11, R13, and R4's severity reduced), one escalated (R12), and one new forward-looking risk (R14, the model-interface gap). Full detail in the Updated Risk Register tab.

11. Unresolved Issues Requiring Track Attention

- ML Engineering / Data Science: the model interface format (DEP-011/ISS-02) needs definition before Phase 5 pipeline integration scoping can begin.
- IT: the capacity conflict (DEP-009/R12) remains unresolved and is now with the Sponsor for a resourcing decision.

13. Week 7 Testing and Refinement Coordination Plan

Week 7 moves the project into Testing → Refinement → End-to-End Validation. Project Management's coordination focus will be:

- Confirm the Sponsor's resourcing decision on the IT Lead conflict (DEP-009) before Phase 7 deployment work depends on it.
- Check in with Data Science on model comparison progress so the model interface (DEP-011) can be defined before it becomes a blocker.
- Align Data Analytics, Data Science, ML Engineering, and Generative AI's individual Week 7 testing plans against the shared schedule, so no track's testing window conflicts with another's.
- Update the Risk and Dependency Registers again at the Week 7 checkpoint, consistent with the pattern established this week.

Part 3 - Mandatory Cross-Track Integration

Two separate integration activities were completed this week. Each is documented against a single, specific track - not a combined or vague track group.

Integration Activity 1 - Data Analytics

Field	Response
1. Track collaborated with	Data Analytics
2. Project dependency	DEP-002 (patient identifier) and risk items R1/R11 (data-quality severity)
3. Information/output received	Data Analytics' Week 6 validated findings confirming the data-quality issue is real but manageable, and confirmation that no clean shared patient identifier exists across scheduling and reminder records
4. Information/output provided	An updated Risk Register entry (R1 downgraded, R11 closed) and an updated Dependency Register entry (DEP-002 moved to Mitigated), plus a Decision Log entry recording the adopted fallback identifier approach
5. Integration activity completed	Reviewed Data Analytics' validated findings against the Week 5 risk assessment, then formally downgraded R1, closed R11 into it, and adopted the fallback linking key as the resolution path for DEP-002
6. What changed as a result	R1 moved from High to Medium likelihood; R11 closed; DEP-002 moved from At Risk to Mitigated; Phase 3 data preparation work can proceed without an open identifier question
7. Evidence	Updated Risk Register (R1, R11); Integration/Dependency Register (DEP-002); Decision Log entry DEC-02; Coordination Record entry, HealthConnect_Week6_Integration_Registers.xlsx

Integration Activity 2 - Generative AI

Field	Response
1. Track collaborated with	Generative AI
2. Project dependency	DEP-004 (Sponsor-approved Clinic Knowledge Base scope for the patient information assistant)
3. Information/output received	Generative AI's Week 6 safety and scope testing evidence, showing the assistant's responses stay within the approved Knowledge Base and escalate correctly on unsupported requests
4. Information/output provided	A formal review of that evidence against the Quality Management Plan's acceptance criteria, and a Decision Log entry confirming the dependency is closed
5. Integration activity completed	Assessed the submitted safety/scope test results against the Quality Management Plan's defined evidence requirements for this dependency, then formally closed DEP-004 rather than leaving it as In Progress

Field	Response
6. What changed as a result	DEP-004 moved from In Progress to Complete; Risk R4 (assistant producing out-of-scope or unsafe responses) had its likelihood downgraded from Medium to Low; assistant design work can proceed without an open governance item
7. Evidence	Integration/Dependency Register (DEP-004); Updated Risk Register (R4); Decision Log entry DEC-04; Coordination Record entry, HealthConnect_Week6_Integration_Registers.xlsx

Integration Validation

Answered once per integration activity, each supported by evidence.

Data Analytics

- What did I have at the end of Week 5? An open risk (R1/R11) flagging that appointment-data quality might be worse than the Charter-stage assumption, and an At Risk dependency (DEP-002) over whether a usable patient identifier exists.
- What did I improve in Week 6? Turned an unvalidated suspicion into a confirmed, scoped finding with an adopted resolution path.
- Which other track did I work with? Data Analytics.
- What did I receive? Their Week 6 validated findings confirming the data-quality issue is real but manageable, and confirmation that no clean shared identifier exists.
- What did I provide? An updated Risk Register and Dependency Register reflecting those findings, and a logged decision (DEC-02) adopting the fallback identifier approach.
- What changed because of the integration? R1 downgraded, R11 closed, DEP-002 moved from At Risk to Mitigated.
- How does this improve the overall HealthConnect project? Phase 3 data preparation can proceed on a resolved, documented basis instead of an open question that could have resurfaced mid-build. Evidence: Updated Risk Register, Dependency Register DEP-002, Decision Log DEC-02.

Generative AI

- What did I have at the end of Week 5? An In Progress dependency (DEP-004) - the Knowledge Base scope for the patient assistant was not yet formally approved.
- What did I improve in Week 6? Converted an open governance item into a closed, evidenced one.
- Which other track did I work with? Generative AI.
- What did I receive? Their Week 6 safety and scope testing evidence, showing the assistant stays within the approved Knowledge Base and escalates correctly.
- What did I provide? A formal review of that evidence against the Quality Management Plan's acceptance criteria, and a logged decision (DEC-04) closing the dependency.
- What changed because of the integration? DEP-004 moved to Complete; Risk R4 downgraded from Medium to Low likelihood.
- How does this improve the overall HealthConnect project? Assistant design work can now proceed without an unresolved safety/scope question hanging over it. Evidence: Dependency Register DEP-004, Updated Risk Register R4, Decision Log DEC-04.

Part 4 - Assumptions, Limitations, Risks and Dependencies Review

Resolved

- The patient-identifier assumption - resolved via the adopted fallback linking key (DEC-02, with Data Analytics).
- The Knowledge Base scope dependency - resolved via accepted Generative AI evidence (DEC-04, with Generative AI).
- The schedule baseline question - resolved via Sponsor sign-off (DEC-01).

Remain Unresolved

- The IT Lead capacity conflict (R12/DEP-009) - escalated but not yet resolved.
- Internal team capacity post-handover - still to be confirmed ahead of Phase 8.

New Issues Discovered in Week 6

- The Data Science → ML Engineering model interface is not yet defined (DEP-011/R14) - not urgent yet, but flagged before it becomes a blocker in Phase 5.

New Cross-Track Dependencies

- DEP-011 (model interface) was identified this week through the Workstream Integration Map exercise itself (Task 1) - a direct product of mapping how Data Science and ML Engineering connect.

Potential Impact and Recommended Mitigation

- If the IT Lead conflict is not resolved before Phase 7, deployment support could be delayed - recommend the Sponsor decide within the next two weeks so alternative resourcing can be arranged if needed.
- If the model interface remains undefined much longer, it could compress ML Engineering's available time in Phase 5 - recommend Data Science prioritise finalising a candidate model interface early in their next work cycle.